PHINS — Regulatory Pre-Ruling Meeting Brief (12 October 2026) · Mr. Avi Ovadia, Capital Market, Insurance & Savings Authority, Jerusalem

Internal · Confidential — preparation brief for the 60-minute pre-ruling meeting with the Capital Market, Insurance & Savings Authority. Do not share internal drafts before the meeting. Every market figure below is tied to a named source. Where two sources disagree, both are shown. Nothing in this brief is a PHINS forecast of National Insurance cash flows or of the health-fund funds.

Meeting facts

Item	Detail
Date	12 October 2026
Duration	60 minutes (hard timebox)
Counterparty	Capital Market, Insurance & Savings Authority
Regulator contact	Mr. Avi Ovadia
Location	Jerusalem — the Authority (the Authority's published letterhead address is 4 Am Ve'Olamo Street, Jerusalem 9546304)
Status	Set
Pack	Pitch dashboard — Authority Pre-Ruling section, plus this brief in English and Hebrew

Purpose: a focused **pre-ruling** dialogue that expands the Regulatory Meeting 27.7 outline from counsel preparation into a direct regulator discussion. The meeting elaborates the **PHINS insurance product**; PHINS as a **functional MGA** (underwriting, billing, claims, actuary and policy administration) acting as the **managerial and operational bridge** between a designated Israeli insurance company and an investment company (for the savings add-on risk — counterparties TBA); **distribution** (direct sales, collaborations, or agents); **adjustable actuarial methods**; **future steps** (technology, policy, B2B contracts); and the **future option** to expand to other markets and become an insurance company.

Goal: pre-agree a **regulation path that approves all PHINS assumptions** and open a standing discussion with the regulator on building future products.

The market context for that ask is the Authority's own 4 October 2026 proposal for the health-fund long-term-care layer, read together with the State Comptroller's June 2026 special report. The proposal is not enacted law. The pre-ruling we ask for is a classification and operating-posture decision on a contract PHINS has already specified.

Elevator pitch — 30 seconds

PHINS is a **built, production insurance platform** asking the Authority for a **pre-ruling** that approves its operating assumptions: the insurance product, the **functional MGA** bridge between a designated Israeli insurance company and an investment company (savings add-on, TBA), three distribution channels, and adjustable actuarial methods — with a staged future option to expand abroad and become an insurance company. Eight days before this meeting the Authority published a permanent-solution proposal that moves younger health-fund insureds toward long-horizon savings plus insurance risk borne by an insurer, and keeps older insureds on today's mechanism as a closed group. The filed PHINS contract is a pure-risk life policy with a lifelong permanent 3+ ADL lump sum, with any savings add-on excluded from the filing. We want the path agreed and a standing dialogue on future products.

Why 12 October 2026 is the right day to ask

Three public facts set the calendar. None of them is a PHINS projection.

1. **The health-fund arrangement the Comptroller examined is temporary through December 2026.** The State Comptroller's special report on Israel's long-term-care insurance array (Tammuz 5786 / June 2026, part of the population-ageing report) records that, after the erosion of Clalit Health Services' insureds' fund, the insurer agreed to keep operating Clalit's long-term-care cover only until the end of 2026, and that the solution eventually reached is temporary until December 2026 only.
2. **On 4 October 2026 the Authority published the permanent-solution proposal** that replaces cross-generational subsidy with a built-in combination of personal accumulation and insurance. Contemporaneous reporting (Funder, Calcalist, TheMarker, Globes; ICE on 5 October) puts the health-fund book at about **5.2 million** insured people. This pack has not attached an Authority PDF of that publication. The 5.2 million figure is used only as those outlets report it.
3. **The meeting sits inside that window:** 12 October 2026, in Jerusalem, eight days after the publication and before the December 2026 expiry the Comptroller describes.

The ask that follows from those facts is narrow. PHINS is not proposing to operate the closed older group, to reinsure the insureds' funds, or to replace the National Insurance nursing benefit. The ask is a pre-ruling on a specified optional private contract for people who can be underwritten into the younger design the Authority has itself described.

What the enclosed 4 December 2023 circular actually changed

The circular supplied with this briefing is **Insurance Circular 2023-1-11**, "Amendment of the consolidated circular — Gate 6, Chapter 3 — Long-term care insurance", dated 4 December 2023 (21 Kislev 5784), signed by

Amit Gal, then acting Commissioner of the Capital Market, Insurance and Savings.

Its operative instruction is short. The 2016 health-fund long-term-care circular had required that, throughout the insurance period, the insurer bear **at least 20 percent** of the insurance risk on the **basic layer**. Circular 2023-1-11 **repeals that minimum**, so a health fund can negotiate the insurer's risk share. It applies to health-fund long-term-care contracts concluded or renewed from publication.

The State Comptroller (June 2026) records the consequence of that December 2023 circular and of the related stabilisation steps:

- The Authority reduced the monthly indemnity by about **10 percent, from 5,500 NIS to 5,000 NIS**, and cancelled the insurers' risk, so that **100 percent** of benefits are paid from the insureds' fund.
- After no insurer bid for Clalit's operator tender, the Authority changed the qualifying test **from difficulty in 3 of 6 ADL to difficulty in 4 of 6 ADL**, and cancelled the insurers' continuation obligation.
- Policies are still marketed in a way that can lead the public to think private insurers bear the risk. After the December 2023 circular, they do not. The insureds bear it. The insurer is described by the Comptroller as managing the fund for a fee.
- At young ages the expected result is negative (annual premiums exceed expected annual receipts). At older ages it is positive. The Comptroller calls this a material cross-subsidy, and states that if a large enough share of younger insureds leave, the funds empty.
- Fund balances are not published to the public.

Funder's 4 October 2026 account, attributing the 2023–2024 steps to the Authority, describes a reduction **from about 6,100 NIS to 5,000 NIS**, an update of ADL definitions, and a higher action threshold, and says the Authority treats those steps as **temporary**. This brief does not average 6,100 and 5,500. Both statements are cited. The direction is the same: a lower indemnity, a tighter ADL test, and insurer risk removed from the basic layer.

That is the model on the enclosed circular. It is the model the Authority, on 4 October 2026, described as structurally unstable over time.

What was published on 4 October 2026 — and what was not decided

Funder (4 October 2026, Ronit Morgenstern), writing up the Authority's publication, states the Authority's account of the current model in these terms: a uniform mutual policy at the health funds, operated by insurers who do not bear the risk; the risk sits entirely on the mutual insureds' fund; the product is voluntary and depends on cross-generational subsidy; there is no central balancing mechanism, so fund size and premium differ across health funds; regulation is split; transparency is weak; policy terms have been changed repeatedly. The Authority's proposed permanent solution is a **built-in combination of insurance and savings**, analogous to the pension system, for about **5.2 million** existing health-fund insureds.

The same account sets the cohort rule without printing an integer:

- An **age threshold will be set**.
- Above it, insureds are a **closed group**. Their rights and continuity of cover stay on the **existing insurance structure**.
- Below it, insureds are spread across insurers by **competitive tender**, with **long-term accumulation** and stability of premium and cover.
- About **85 percent** of long-term-care claims occur **above age 70**, and the chance of becoming nursing-dependent rises from that age.
- Care cost is described as about **10,000 NIS a month at home** and about **15,000 NIS a month in an institution**, for an average spell of about **40 months**.
- Legislation is required.

Other outlets the same week add details that Funder does not, and they do not all agree. This table is the whole of what this pack relies on. A cell is blank where that outlet did not state the point.

Point	Funder, 4 Oct 2026	Calcalist, 4 Oct 2026, 19:06	TheMarker, 4 Oct 2026, 06:15	Globes (same publication cycle)	ICE, 5 Oct 2026
Book size	about 5.2 million health-fund insureds	about 5.2 million, to be split	5.2 million; described as the largest cover of its kind	about 5.2 million of about 6.7 million adults aged 18+; about 2.7 million via Clalit	about 5.2 million
Age cut	a threshold will be set; not printed	**55**. Age 55 and over: closed group	not printed in the report we read	a threshold will be set	older closed group / younger cohort; integer not printed
Who runs the closed group	existing insurance structure; rights preserved	a new statutory corporation Calcalist names **Siudit**	—	existing policy form, to protect rights	existing form; rights and continuity preserved
Younger cohort	competitive tenders; long-term accumulation; insurers	long-term accumulation at insurers chosen by tender; insurers bear the insurance risk	personal long-term-care savings, pension-like, each person saving until about age 70	personal accumulation from a young age; savings use discussed around age 70 because about 85 percent of claims are above 70	controlled split across insurers by tender; personal accumulation
Legal status	legislation required	Budget Division is expected to oppose; the next finance minister decides	Finance Ministry disagrees and considers the model stable enough after recent tightenings; the next government decides	to be put before the next government; voluntary shelf product first, a mandatory contribution possibly later	principles already shown to professional government staff; legislation required, so not immediate
Benefit the new model is described as aiming to support	—	—	—	about 5,000 NIS/month at home and about 10,000 NIS/month in an institution, for up to five years, from an accumulation estimated at about 300,000–600,000 NIS	—

Two corrections belong in the spoken brief, because they are easy to overstate.

The age cut is not 50, and it is not yet a statute. A December 2025 staff outline reported by Bizportal discussed a band of roughly under 50 to 60. That article predates the 4 October 2026 publication and is not the publication. Calcalist is the outlet that prints **55**. Funder, Globes and ICE say the integer will be set. The question for Mr. Ovadia is which integer the Authority will defend, and whether 55 (Calcalist) is the working figure. PHINS does not need him to adopt 50. The filed contract is issuable below age 65. The disability layer in the filing attaches from age 3. There is no issue cap at 50.

The proposal is not destined, and it is not in force. TheMarker and Calcalist both report that the Finance Ministry side does not share the Authority's urgency. Calcalist reports that the Budget Division considers the health-fund model to have stabilised after the recent cuts. Any pre-ruling conversation should acknowledge that disagreement rather than speak as if the Knesset had already passed the split.

Clalit's book is stated two ways. The Comptroller (June 2026) says Clalit insures **about 2.5 million** people in this cover. Globes, reporting the 2026 book, says **about 2.7 million**. Both figures are left in the record. Approved health-fund long-term-care claims, on the Comptroller's series, rose from **8,195 to 20,711** between 2012 and 2023.

National Insurance — what the Comptroller measured

The National Insurance Institute (Bituah Leumi) nursing benefit is the public layer. It is not the health-fund policy, and it is not the PHINS contract. The Comptroller's finding is that the public layer and the health-fund layer are both under long-term strain, for different reasons.

Figures below are from the June 2026 special report unless another source is named.

Measure	Figure	Source note
Nursing-benefit recipients	about 180,000 in 2018, on the eve of the nursing reform; about 392,000 in 2025	more than double; the share of retirement-age residents rose from about 16 percent to about 30 percent, which the Comptroller describes as more than double the OECD average
Annual nursing expenditure	about 7 billion NIS in 2018; about 21.1 billion NIS in 2025 in the report's opening statement (the findings text rounds 2025 to about 21 billion)	the increase is about 14.1 billion NIS. The Budget Division's 2018 estimate of the reform's annual add-on was about 1.3 billion NIS; the Comptroller records an actual annual add-on of about 14 billion NIS
Fund-zero year	2035, as of the close of the audit	the Institute's 2024 full actuarial report had brought the year forward 8.6 years, from 2044 to 2036, of which 6.3 years were attributed to the nursing-benefit changes. By the close of the audit the year stood at 2035. A December 2025 Institute estimate, cited by the Comptroller, is the same year: a cash deficit in which, without further funding, the Institute cannot pay its statutory obligations in full
2026 budget	the 2024, 2025 and 2026 Institute budgets were deficit budgets, with a forecast deficit of about 10 billion NIS in 2026	the socio-economic cabinet did not discuss the balance, contrary to the 2015 government decision the Comptroller cites
Assessment method	Israel is described as the only OECD country that systematically assesses dependency without a face-to-face meeting; about 1 percent of assessments are in the elder's home	document-based assessments were approved at a rate about 16 percentage points higher than in-home assessments
Gap versus a health-fund ADL assessment	the Institute's dependency level was about 1.7 levels higher; the Comptroller estimates the excess cost at about 9.5 billion NIS a year	in about 75 percent of the compared assessments the Institute's level was higher. The health-fund assessments in the comparison were in person
Cash-flow warning, separate from the fund-zero year	a Finance Ministry representative told the Knesset on 29 June 2026 that the actuarial deficit is acute and that, on the Ministry's analysis, 2029 is already the point at which the cash-flow ability to pay statutory benefits is in question. A decade earlier the zero year had been put around 2045	Knesset press release, 29 June 2026. This is a Treasury statement about payment capacity. It is not the Comptroller's 2035 fund-zero year, and the two should not be collapsed into one sentence

The honest sentence for the room is this. The Institute's own actuarial path, as examined by the Comptroller, brings the reserve fund to zero in **2035**, with a 2026 deficit budget of about **10 billion NIS** already in the audited

record, and with a Treasury warning that statutory payment capacity can come under pressure from **2029**. That is a negative actuarial balance that worsens because long-term nursing settlements have grown far beyond the 2018 estimate. It is not a finding that the Institute is insolvent in October 2026, and it is not a finding that citizens who meet a 3+ ADL test have no benefit today. The nursing benefit still exists. The health-fund indemnity still exists, on a tighter 4-of-6 test, through a temporary arrangement the Comptroller dates to December 2026. The Comptroller's forward sentence is that, looking at the expected growth in the number of nursing-dependent elderly, there is a concern that the long-term-care array does not have a sustainable ability to provide what residents need when they become nursing-dependent.

The household gap sits beside that public finding. On the Authority's cost figures as reported by Funder (about 10,000 NIS at home and about 15,000 NIS in an institution), a 5,000 NIS health-fund indemnity — the Comptroller's post-cut monthly figure — leaves a large residual with the family for a spell of about 40 months. Globes reports that the savings model is aimed at reproducing about 5,000 / 10,000 for up to five years, which still sits below the 10,000 / 15,000 cost pair. PHINS does not adopt those costs as its own pricing assumptions.

Where the filed PHINS contract fits

The contract in the Israel regulatory application memorandum (Hebrew canonical, Draft 3.1; English companion; Section 15 is the 12 October 2026 supplement and leaves Sections 1–5 unchanged) is a **pure-risk** life policy. It has **no savings component, no cash value, and no surrender value**. An optional savings add-on exists on the platform and is **excluded from the filing**. If it is offered in Israel it is to be filed separately through a properly licensed entity. That exclusion is an advantage in this meeting, because the Authority is now talking about savings and insurance together, and the filing already refuses to hide savings inside the risk policy.

Filed term (Draft 3.1)	What it is
Life sum	Face amount F below age 65; F divided by 4 from age 65. Maximum face below 65 is 1,000,000 NIS
Disability sum	A single lump sum. Below 65 it is life divided by 4, so F divided by 4 (contract ratio 1:4), capped at 250,000 NIS. From 65 it equals the stepped-down life sum (contract ratio 1:1 at that reduced sum)
Worked example	F = 500,000 NIS. Below 65: life 500,000, disability 125,000. From 65: life 125,000, disability 125,000
Trigger	Permanent inability to perform at least 3 Activities of Daily Living, or an equivalent cognitive impairment, confirmed by medical and functional assessment. The paid benefit is binary: once the trigger is confirmed, the full disability sum is paid. Internal scores do not grade the payment
After a claim	The policy may continue. Below 65 the life sum becomes 0.75 times F from the next premium due date. The disability layer ends with its single payment
Pricing	One deterministic pricing kernel. The memorandum states that the rate tables are the platform's central tables version V2.0. The Authority's published long-term-care experience tables (annexed to the consolidated circular, Gate 6, Part 3, December 2023 amendment) are a benchmark and a reasonableness reference. They are not copied as the price
Who bears insurance risk	A licensed insurer and/or an approved reinsurance structure. PHINS is the technology and administration layer

The ADL difference must be said aloud. The health-fund basic layer, on the Comptroller's account, was tightened **from 3 of 6 ADL to 4 of 6 ADL**. The filed PHINS trigger remains **permanent 3+ ADL** (or cognitive impairment) and pays a lump sum, not a five-year indemnity. That is a different promise from the tightened health-fund indemnity. It is also the promise the memorandum already asks the Authority to classify as life insurance with a disability mechanism, with a pre-ruling if the Authority reads the layer as a health or

long-term-care class.

The savings difference must be said aloud as well. The Authority's younger-cohort proposal, as reported, is personal accumulation plus an insurance component, with the insurer bearing risk. The filed PHINS policy is the insurance component only. The platform's savings add-on is the separate piece, and the counterparties for that piece remain TBA. Asking the Authority to treat them as one product would contradict the filing.

The pre-ruling we are in a position to request

The memorandum itself says it is a drafting framework and not a substitute for a pre-ruling. Section 40 of the Supervision of Financial Services (Insurance) Law, 5741-1981, is the approval gate the memorandum is built for. The filing list already specified there — appointed-actuary pricing report, morbidity and mortality by age, lapse, expense and commission, reinsurance and solvency, sensitivity cases, the consumer premium table, and a documented explanation of any deviation from the Authority's long-term-care benchmark — is the submission, not a new list invented for this meeting.

What changed on 4 October 2026 is the reason the classification question is now urgent, not the contract.

Ask Mr. Ovadia for five decisions, each one something the platform can already show:

1. **Classification.** A pre-ruling that the Draft 3.1 contract is life insurance with a lifelong permanent-disability mechanism under Section 40, and that the permanent 3+ ADL lump sum does not by itself reclassify the policy as health-fund group long-term care.
2. **Risk and the functional MGA.** Confirmation that PHINS may perform underwriting, billing, claims administration and actuarial support as the managerial and operational bridge, while claims reserves and final adjudication stay on a designated Israeli insurance company, and while the carrier's appointed actuary signs the basis. Every assumption change on the platform is versioned. AI recommends; the ledger decides; a person supervises.
3. **Savings, separately.** Direction on the licence, the entity, and the sequence if PHINS later files the savings add-on for the younger cohort the Authority has described. The add-on is not part of this pre-ruling. Counterparties are TBA. No counterparty is named in this brief.
4. **Perimeter.** Confirmation that this pre-ruling does not appoint PHINS to operate the closed older group, the insureds' funds, or the National Insurance nursing benefit. Those books stay where the statute puts them.
5. **Path and clock.** The submission order, the open conditions, named contacts, and a date for the next meeting. The Comptroller's December 2026 horizon is the reason to ask for a clock. It is not a reason to claim the Authority has already chosen PHINS.

Distribution (direct digital sales, collaborations, licensed agents), adjustable actuarial methods, and the later option to expand to other markets and become an insurance company stay on the agenda as future steps. They are not the decision this meeting has to finish.

60-minute run of show — hard timeboxes

Minutes	Segment	Talking points & source
00–05	Opening — PHINS in five minutes	AI-operated insurance operating system, **159 deployed modules** (live count from the `services/` layer) on an append-only hash-chained ledger — **AI recommends, the ledger decides**. State the goal up front: a pre-ruling path that approves all PHINS assumptions. Name the day: 12 October 2026, Jerusalem, eight days after the Authority's publication.

Minutes	Segment	Talking points & source
05–15	PHINS insurance product — pre-ruling item 1	Life insurance with a lifelong disability mechanism (1:4 · 1:1 from 65) per the published **Israel regulatory application memorandum (Hebrew)**; the exact product assumptions we ask the Authority to confirm in the pre-ruling. Say the ADL difference once: the health-fund layer was tightened to 4 of 6; the filing is permanent 3+ ADL, lump sum, pure risk.
15–25	Functional MGA — the operational bridge	PHINS operates **underwriting, billing, claims, actuarial and policy administration** as the managerial and operational bridge between a **designated Israeli insurance company** (risk carrier — claims reserves stay on the carrier) and an **investment company** for the savings add-on risk (counterparties TBA). Governance: every decision ledger-logged and auditable by the Authority.
25–32	Distribution	**Direct digital sales**, **collaborations** (bancassurance-style and affinity partners), and **licensed agents** — the licensing and supervision posture per channel, and how the platform enforces suitability and disclosure identically across all three.
32–40	Adjustable actuarial methods	Pricing and reserving methods that adjust with emerging experience; every assumption change versioned and ledger-logged with regulator visibility; appointed-actuary sign-off preserved on the carrier side. One kernel prices the contract; the December 2023 long-term-care tables are a benchmark, not a copied tariff.
40–48	Future steps — technology, policy, B2B contracts	Technology roadmap (AI supervision model, reporting interfaces for the Authority), policy wording development, and the B2B contract stack: carrier agreement, investment-company agreement (savings add-on, TBA), and distributor agreements.
48–55	The public record, then the perimeter	Two minutes on Circular 2023-1-11 and the 4 October proposal (5.2 million, closed group, younger accumulation, legislation still required, age integer not uniform across reports). Two minutes on the Comptroller: expenditure about 7 to about 21.1 billion, recipients about 180,000 to about 392,000, fund-zero year 2035, temporary health-fund solution until December 2026. Then the perimeter: PHINS is not bidding to run the closed group or the nursing benefit.
55–60	Wrap-up & the pre-ruling path	Agree the pre-ruling route: submission list, open questions, named contacts on both sides, timeline — and a standing open discussion with the regulator on building future products. Log the outcome in the Meeting Diary.

Run-of-show rules: hard timeboxes with a visible timer, one speaker per segment, materials pre-shared, demo pre-loaded, decisions captured live in the diary — questions parked to the wrap-up unless blocking. If a figure is challenged, read the source cell. Do not upgrade Calcalist's 55 into "the Authority decided 50". Do not upgrade 2035 into "insolvent now".

Functional MGA — who holds what

The structure we ask the Authority to pre-rule: PHINS runs the operating functions; the designated Israeli insurance company carries the risk; an investment company carries the savings add-on risk (counterparties TBA).

Function	PHINS (functional MGA)	Counterparty
Underwriting	AI severity underwriting within carrier-approved guidelines; humans on flagged edge cases.	Designated Israeli insurance company approves guidelines and holds the risk.
Billing	Automated billing, auto-pay and dunning on the platform ledger.	Premium flows reconciled to the carrier; savings component to the investment company (TBA).
Claims	AI triage + fraud detection; human adjudication on complex or contested claims.	Claims reserves stay on the carrier; carrier retains final adjudication authority.
Actuary	Adjustable pricing / reserving models; every assumption change versioned and ledger-logged.	Carrier's appointed actuary signs off; Authority has audit visibility.
Savings add-on	Platform administration and unified customer view. Excluded from the Section 40 filing.	Investment company (TBA) carries the savings add-on risk under its own licence.

So what: PHINS is the managerial and operational layer — the risk stays on a licensed carrier — so the pre-ruling can approve the platform posture without transferring insurance risk to an unlicensed entity. The future insurance-company option is a separate, staged decision. The same separation is what lets the Authority look at a pure-risk 3+ ADL contract now, and at a savings add-on later, which is the sequence the 4 October proposal itself implies for any private product offered to the younger cohort.

Pre-meeting preparation (send / load before the meeting)

- **This brief**, English and Hebrew.
- **Israel regulatory application (Hebrew)** — `/investor-docs/israel-regulatory-application-he.pdf` (English version available).
- **Pre-ruling assumption list** — the enumerated PHINS assumptions we ask the Authority to approve (product, functional MGA, distribution, actuarial methods), plus the five decisions in this brief.

- **Functional MGA bridge one-pager** — who holds which function and which risk (carrier vs investment company vs PHINS).
- **PHINS business plan** — canonical IL model, future steps, and multi-market roadmap.
- **Insurance Circular 2023-1-11** (4 December 2023) and the State Comptroller special report (June 2026), so every external figure can be opened if it is challenged.
- Platform demo pre-loaded (pitch dashboard + admin portal) — shown only if time allows; the timeline never slips for it.

Decision outputs — leave the meeting with

1 An agreed **pre-ruling path**: what to submit, in what order, and the Authority's expected timeline, against the December 2026 horizon the Comptroller recorded for the temporary health-fund arrangement.

1 The Authority's read on each **PHINS assumption** — product, functional MGA bridge, distribution channels, adjustable actuarial methods — approved, conditioned, or open. In particular, the classification of the permanent 3+ ADL lump sum.

1 Named **contacts on both sides** and a standing channel for the open discussion on building future products.

1 The Authority's view on the **future expansion** options — other markets and the staged insurance-company licence — and on the separate licence path for a savings add-on.

1 The Authority's working age integer for the cohort split, recorded as their answer, not as ours.

1 A scheduled follow-up — logged in the Meeting Diary on the pitch dashboard.

Source register

Source	What this brief takes from it
Insurance Circular 2023-1-11, 4 December 2023, Amit Gal, acting Commissioner	Repeal of the minimum 20 percent insurer risk share on the health-fund basic layer. Read from the circular supplied for this briefing.

Source	What this brief takes from it
State Comptroller, special report, long-term-care insurance array in Israel, Tammuz 5786 / June 2026	180,000 to 392,000 recipients; about 7 to about 21.1 billion NIS; fund-zero year 2035; about 10 billion NIS deficit forecast for 2026; 5,500 to 5,000 NIS; 3-of-6 to 4-of-6 ADL; 100 percent of benefits from the insureds' fund; Clalit about 2.5 million; temporary operation until the end of 2026; claims 8,195 to 20,711 (2012–2023); +16 percentage points; 1.7 levels and about 9.5 billion NIS; cross-subsidy by age.
Knesset press release, 29 June 2026	Finance Ministry statement: actuarial deficit acute; 2029 as the cash-flow pressure point for statutory payments; zero year around 2045 a decade earlier.
Funder, 4 October 2026	Authority publication: structural failure of the mutual model; insurance plus savings; 5.2 million; age threshold to be set; closed group; competitive tenders; 85 percent of claims above 70; care cost about 10,000 / 15,000; average spell about 40 months; 2023–2024 steps from about 6,100 to 5,000 described as temporary; legislation required.
Calcalist, 4 October 2026	5.2 million; age 55; Siudit; Budget Division expected to oppose; next finance minister.
TheMarker, 4 October 2026	Model not stable long-term; savings until about age 70; Finance Ministry disagrees; next government; 5.2 million.
Globes, same publication cycle	6.7 million adults 18+; 5.2 million insured; about 2.7 million via Clalit; threshold to be set; accumulation about 300,000–600,000 NIS; target benefit about 5,000 / 10,000 for up to five years.
ICE, 5 October 2026	Same direction; legislation; 85 percent; costs; about 40 months.
Bizportal, December 2025	Earlier outline only. A band of roughly 50 to 60. Not the 4 October publication.
Israel regulatory application memorandum, Draft 3.1, Section 15	The contract terms in the fit table, plus the 12 October 2026 supervision supplement. Hebrew canonical, English companion. Sections 1–5 are unchanged.
PHINS business plan	Canonical IL planning model. This brief does not restate a valuation.

Data integrity notice: this brief is an operational admin preparation tool. Market figures are copied from the sources named above and are not PHINS estimates. The 4 October 2026 proposal is a published recommendation that still requires legislation; the Finance Ministry side has disputed its urgency in the same day's reporting. The age integer is unresolved in the public record except for Calcalist's report of 55. National Insurance's fund-zero year in the Comptroller's report is 2035; the Institute is not described here as insolvent on the meeting date. The designated Israeli insurance company and the investment company for the savings add-on are TBA — no counterparty is named or committed. The regulatory product content derives from the published Israel regulatory application memorandum and the PHINS business plan. The agenda, prep list, and decision outputs are internal planning aids — not legal advice, not regulatory commitments — and this document does not write to the platform ledger.